

ARTWORK DETAIL LABEL

Product	Oralcon				
Buyer/Country	Malaysia				
Item Code	XXXXXXXX				
Dimension	Open Size: 134 x 223 mm, Folding size : 134 x 28 mm	Component	Leaflet	Pack	NA
Colour Shades	Black			No. of Colours	1
Non Printing Colour	Die Line				
Date	30-12-2024				
Special Instructions	NA				

FRONT

BACK

Oralcon

Levonorgestrel and Ethinylestradiol Tablets
21 Tablets

Composition

Each sugar coated white tablet contains
Levonorgestrel Ph. Eur. 0.15 mg
Ethinylestradiol Ph. Eur. 0.03 mg
Colour: Titanium Dioxide

Pharmaceutical Form

Sugar coated tablet,
Tablet is white, circular, biconvex.

Clinical Pharmacology

Oralcon is an oestrogen-progestogen combination which acts by inhibiting ovulation by suppression of the mid-cycle surge of luteinising hormone, the inspisation of cervical mucus producing a barrier to sperm, and the rendering of the endometrium unresponsive to implantation.

Pharmacokinetic properties

Levonorgestrel

Levonorgestrel is absorbed quickly and completely. Maximum active substance levels of approx. 3 ng/ml were reached in serum just one hour after ingestion of Oralcon. The serum concentrations subsequently fell in 2 phases with half-lives of around 0.5 hours and 20 hours. The metabolic clearance rate from plasma is approx. 1.5 ml/min/kg.

Levonorgestrel is eliminated not in unchanged form, but in the form of metabolites with a half-life of around one day and in almost equal proportions via the kidney and bile. Biotransformation takes place via the familiar pathways of steroid metabolism. There are no known pharmacologically active products of metabolism.

Levonorgestrel is bound to serum albumin and SHBG (Sex Hormone Binding Globulin). Only around 1.5% of the respective total concentration is present in unbound form, while approx. 65% is bound to SHBG. The relative proportions (free, albumin-bound, SHBG-bound) depend on the concentration of SHBG. After induction of the binding protein, the portion bound to SHBG increases, while the free portion and that bound to albumin decreases.

After daily repeated ingestion, Levonorgestrel accumulates by about the factor 2. A steady state is reached during the second half of the treatment cycle. The pharmacokinetics of Levonorgestrel are dependent on the concentration of SHBG in plasma. Under treatment with Oralcon, an increase in the serum levels of SHBG effect a concomitant increase in the specific binding capacity and therefore also an increase in Levonorgestrel serum levels.

The Levonorgestrel serum levels do not change any further after 1 - 3 cycles of use owing to the fact that SHBG induction is concluded. Compared to a single administration, 3 - 4 fold higher Levonorgestrel serum levels are reached in the steady state.

The absolute bioavailability of Levonorgestrel amounts to almost 100%.

Approx. 0.1% of the maternal dose can be passed on to a baby with the breast milk.

Ethinylestradiol

Orally administered Ethinylestradiol is absorbed quickly and completely. Ingestion of Oralcon leads to maximum plasma levels of approx. 100 pg/ml after 1 - 2 hours. The substance concentration then falls in 2 phases for which half-lives of around 1 - 2 hours and about 20 hours have been determined. For technical reasons, these data can only be calculated at higher dosages.

An imaginary distribution volume of around 5 l/kg and a metabolic clearance rate from plasma of approx. 5 ml/min/kg have been determined for Ethinylestradiol. Ethinylestradiol is bound non-specifically to serum albumin to the extent of 98%.

Ethinylestradiol is metabolised even during its absorption phase and during its first liver transit, reduced and individually varying oral bioavailability. Ethinylestradiol is eliminated not in unchanged form, but in the form of metabolites with a half-life of around one day. The excretion ratio is 40 (urine): 60 (bile).

Because of the half-life of the terminal elimination phase from plasma, a steady state characterised by a 30 - 40% higher plasma substance level becomes established after approx. 5-6 daily administrations.

The absolute bioavailability of Ethinylestradiol is subject to considerable interindividual variations. After oral ingestion, it amounts to around 40 - 60% of the dose.

In women with fully established lactation, around 0.02% of the maternal dose can be passed on to the baby with the breast milk.

Other drugs can have a negative or positive effect on the systemic availability of Ethinylestradiol. No interaction with vitamin C takes place. On continuous use, Ethinylestradiol induces the hepatic synthesis of CBG and SHBG, the extent of SHBG induction being dependent on the type and dose of the simultaneously administered progestogen.

Indication

Oral contraception.

Contraindications

- Pregnancy.
- Severe disturbances of liver function, jaundice or persistent itching during a previous pregnancy, Dubin-Johnson syndrome, Rotor syndrome, previous or existing liver tumours.
- Existing or a history of confirmed venous thromboembolism (VTE), family history of idiopathic VTE and other known risk factors for VTE.
- Existing or previous arterial thrombotic or embolic processes.
- Conditions which predispose to thromboembolism e.g. disorders of the clotting processes, valvular heart disease and atrial fibrillation.
- Stroke-cell anaemia.
- Mammary or endometrial carcinoma or a history of these conditions.
- Severe diabetes mellitus with vascular changes.
- Disorders of lipid metabolism.
- History of herpes gestationis.
- Deterioration of otosclerosis during pregnancy.
- Undiagnosed abnormal vaginal bleeding.
- Hypersensitivity to any of the components of Oralcon.
- Oralcon is contraindicated for concomitant use with the medical products containing ombitasvir/ paritaprevir/ ritonavir and dasabuvir (See Section Warnings and Precautions and Section Interactions with Other Medicaments).

Special warnings and precautions

- The use of any combined oral contraceptive carries an increased risk of Venous Thromboembolism (VTE) compared with no use. The excess risk of VTE is highest during the first year a woman ever uses a combined oral contraceptive. This increased risk is less than the risk of VTE associated with pregnancy which is estimated as 60 cases per 100 000 pregnancies.

- Some epidemiological studies have reported a greater risk of VTE for women using combined oral contraceptives containing Desogestrel or Gestodene (the so-called 'third generation' pills) than for women using pills containing Levonorgestrel (the so-called 'second generation' pills).
- The spontaneous incidence of VTE in healthy non-pregnant women (not taking any oral contraceptive) is about 5 cases per 100 000 per year. The incidence in users of second generation pills is about 15 per 100 000 women per year of use. The incidence in users of third generation pills is about 25 cases per 100 000 women per year of use; this excess incidence has not been satisfactorily explained by bias or confounding. The level of all these risks of VTE increases with age and is likely to be further increased in women with other known risk factors for VTE such as obesity.

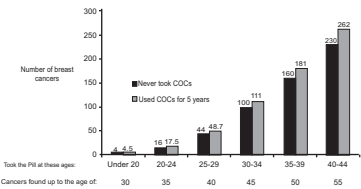
The risk of venous and/or arterial thrombosis associated with combined oral contraceptives increases with:

- age;
- smoking (with heavier smoking and increasing age the risk further increases, especially in women over 35 years of age);
- a positive family history (i.e. venous or arterial thromboembolism ever in a sibling or parent at a relatively early age). If a hereditary predisposition is suspected, the woman should be referred to a specialist for advice before deciding about any COC use;
- obesity (body mass index over 30 kg/m²);
- dyslipoproteinaemia;
- hypertension;
- valvular heart disease;
- atrial fibrillation;
- prolonged immobilisation, major surgery, any surgery to the legs, or major trauma. In these situations it is advisable to discontinue COC use (in the case of elective surgery at least six weeks in advance) and not to resume until two weeks after complete remobilisation.

There is no consensus about the possible role of varicose veins and superficial thrombophlebitis in venous thromboembolism.

- The increased risk of thromboembolism in the puerperium must be considered
- Other medical conditions which have been associated with adverse circulatory events include diabetes mellitus, systemic lupus erythematosus, haemolytic uraemic syndrome, chronic inflammatory bowel disease (Crohn's disease or ulcerative colitis), sickle cell disease and subarachnoid haemorrhage.
- An increase in frequency or severity of migraine during COC use (which may be prodromal of a cerebrovascular event) may be a reason for immediate discontinuation of the COC.
- Biochemical factors that may be indicative of hereditary or acquired predisposition for venous or arterial thrombosis include Activated Protein C (APC) resistance, hyperhomocysteinaemia, antithrombin-III deficiency, protein C deficiency, protein S deficiency, antiphospholipid antibodies (anticardiolipin antibodies, lupus anticoagulant).
- When considering risk/benefit, the physician should take into account that adequate treatment of a condition may reduce the associated risk of thrombosis and that the risk associated with pregnancy is higher than that associated with COC use
- Numerous epidemiological studies have been reported on the risks of ovarian, endometrial, cervical and breast cancer in women using combined oral contraceptives. The evidence is clear that combined oral contraceptives offer substantial protection against both ovarian and endometrial cancer.
- An increased risk of cervical cancer in long-term users of combined oral contraceptives has been reported in some studies, but there continues to be controversy about the extent to which this is attributable to the confounding effects of sexual behaviour and other factors.

- A meta-analysis from 54 epidemiological studies reported that there is a slightly increased relative risk (RR = 1.24) of having breast cancer diagnosed in women who are currently using combined oral contraceptives (COCs). The observed pattern of increased risk may be due to an earlier diagnosis of breast cancer in COC users, the biological effects of COCs or a combination of both. The additional breast cancers diagnosed in current users of COCs or in women who have used COCs in the last ten years are more likely to be localised to the breast than those in women who never used COCs.
- Breast cancer is rare among women under 40 years of age whether or not they take COCs. Whilst this background risk increases with age, the excess number of breast cancer diagnoses in current and recent COC users is small in relation to the overall risk of breast cancer (see bar chart).
- The most important risk factor for breast cancer in COC users is the age women discontinue the COC; the older the age at stopping, the more breast cancers are diagnosed. Duration of use is less important and the excess risk gradually disappears during the course of the 10 years after stopping COC use such that by 10 years there appears to be no excess.
- The possible increase in risk of breast cancer should be discussed with the user and weighed against the benefits of COCs taking into account the evidence that they offer substantial protection against the risk of developing certain other cancers (e.g. ovarian and endometrial cancer).
- The possibility cannot be ruled out that certain chronic diseases may occasionally deteriorate during the use of combined oral contraceptives (see 'Precautions').
- In rare cases benign and, in even rarer cases, malignant liver tumours leading in isolated cases to life-threatening intra-abdominal haemorrhage have been observed after the use of hormonal substances such as those contained in Oralcon. If severe upper abdominal complaints, liver enlargement or signs of intra-abdominal haemorrhage occur, the possibility of a liver tumour should be included in the differential diagnosis.
- Estimated cumulative numbers of breast cancers per 10,000 women diagnosed in 5 years of use and up to 10 years after stopping COCs, compared with numbers of breast cancers diagnosed in 10,000 women who had never used COCs



ALT elevations

- During clinical trials with patients treated for hepatitis C virus infections (HCV) with the medicinal products containing ombitasvir/ paritaprevir/ ritonavir and dasabuvir with/ without ribavirin, transaminase (ALT)

elevations higher than 5 times the upper limit of normal (ULN) occurred significantly more frequent in women using ethinylestradiol-containing medications such as combined hormonal contraceptives (CHCs). Patients who are taking ethinylestradiol-containing medicinal products must switch to an alternative method of contraception (e.g. progestin only contraception or non-hormonal methods) prior to initiating ombitasvir/ paritaprevir/ ritonavir and dasabuvir therapy (See Section Contraindications and Section Interactions with Other Medicaments).

Reasons for stopping oral contraception immediately:

- Occurrence for the first time, or exacerbation, of migrainous headaches or unusually frequent or unusually severe headaches
- Sudden disturbances of vision or hearing or other perceptual disorders
- First signs of thrombophlebitis or thromboembolic symptoms (e.g. unusual pains in or swelling of the leg(s), stabbing pains on breathing or coughing for no apparent reason), Feeling of pain and tightness in the chest
- Six weeks before an elective major operation (e.g. abdominal, orthopaedic), any surgery to the legs, medical treatment for varicose veins or phlebotomy, immobilisation, e.g. after accidents or surgery, Do not restart until 2 weeks after full ambulation. In case of emergency surgery, thrombotic prophylaxis is usually indicated e.g. subcutaneous heparin
- Onset of jaundice, hepatitis, itching of the whole body
- Increase in epileptic seizures
- Significant rise in blood pressure
- Onset of severe depression
- Severe upper abdominal pain or liver enlargement
- Clear exacerbation of conditions known to be capable of deteriorating during oral contraception or pregnancy
- Pregnancy is a reason for stopping immediately because it has been suggested by some investigations that oral contraceptives taken in early pregnancy may slightly increase the risk of foetal malformations. Other investigations have failed to support these findings. The possibility therefore cannot be excluded, but it is certain that if a risk exists at all, it is very small.

Precautions

Assessment of women prior to starting oral contraceptives (and at regular intervals thereafter) should include a personal and family medical history of each woman. Physical examination should be guided by this and by the contraindications and warnings for this product. The frequency and nature of these assessments should be based upon relevant guidelines and should be adapted to the individual woman, but should include measurement of blood pressure and, if judged appropriate by the clinician, breast, abdominal and pelvic examination including cervical cytology.

The following conditions require strict medical supervision during medication with oral contraceptives. Deterioration or first appearance of any of these conditions may indicate that use of the oral contraceptive should be discontinued:

- Diabetes mellitus, or a tendency towards diabetes mellitus (e.g. unexplained glycosuria), hypertension, varicose veins, a history of phlebitis, otosclerosis, multiple sclerosis, epilepsy, porphyria, tetany, disturbed liver function, Sydenham's chorea, renal dysfunction, family history of clotting disorders, obesity, family history of breast cancer and patient history of benign breast disease, history of clinical depression, systemic lupus erythematosus, uterine fibroids and migraine, gall-stones, cardiovascular diseases, chloasma, asthma, an intolerance of contact lenses, or any disease that is prone to worsen during pregnancy.
- Some women may experience amenorrhoea or oligomenorrhoea after discontinuation of oral contraceptives, especially when these conditions existed prior to use. Women should be informed of this possibility.

Undesirable Effects

- In rare cases, headaches, gastric upsets, nausea, vomiting, breast tenderness, changes in body weight, changes in libido, depressive moods can occur.
- In predisposed women, use of Oralcon can sometimes cause chloasma which is exacerbated by exposure to sunlight. Such women should avoid prolonged exposure to sunlight.
- Individual cases of poor tolerance of contact lenses have been reported with use of oral contraceptives. Contact lens wearers who develop changes in lens tolerance should be assessed by an ophthalmologist.
- Menstrual changes:
 - Reduction of menstrual flow: This is not abnormal and it is to be expected in some patients. Indeed, it may be beneficial where heavy periods were previously experienced.
 - Missed menstruation: Occasionally, withdrawal bleeding may not occur at all. If the tablets have been taken correctly, pregnancy is very unlikely. If withdrawal bleeding fails to occur at the end of a second pack, the possibility of pregnancy must be ruled out before resuming with the next pack.
- Intermenstrual bleeding: 'Spotting' or heavier 'breakthrough bleeding' sometimes occur during tablet taking, especially in the first few cycles, and normally cease spontaneously. Oralcon should therefore, be continued even if irregular bleeding occurs. If irregular bleeding is persistent, appropriate diagnostic measures to exclude an organic cause are indicated and may include curettage. This also applies in the case of spotting which occurs at irregular intervals in several consecutive cycles or which occurs for the first time after long use of Oralcon.
- Effect on blood chemistry: The use of oral contraceptives may influence the results of certain laboratory tests including biochemical parameters of liver, thyroid, adrenal and renal function, plasma levels of carrier proteins and lipid/lipoprotein fractions, parameters of carbohydrate metabolism and parameters of coagulation and fibrinolysis. Laboratory staff should therefore be informed about oral contraceptive use when laboratory tests are requested.

Refer to 'Special warnings and special precautions for use' for additional information.

Drug Interactions

- Hepatic enzyme inducers such as barbiturates, primidone, phenobarbitone, phenytoin, phenylbutazone, rifampicin, carbamazepine and griseofulvin can impair the efficacy of Oralcon. For women receiving long-term therapy with hepatic enzyme inducers, another method of contraception should be used. The use of ampicillin and other antibiotics may also reduce the efficacy of Oralcon, possibly by altering the intestinal flora.
- Women receiving short courses of enzyme inducers or broad spectrum antibiotics should take additional, non-hormonal (except rhythm or temperature method) contraceptive precautions during the time of concurrent medication and for 7 days afterwards. If these 7 days overrun the end of a pack, the next pack should be started without a break. In this situation, a withdrawal bleed should not be expected until the end of the second pack. If the patient does not have a withdrawal bleed during the tablet-free interval following the end of the second pack, the possibility of pregnancy must be ruled out before resuming with the next pack. With ritampicin, additional contraceptive precautions should be continued for 4 weeks after treatment stops, even if only a short course was administered.

- The requirement for oral antidiabetics or insulin can change as a result of the effect on glucose tolerance.
- The herbal remedy St John's wort (Hypericum perforatum) should not be taken concomitantly with Oralcon as this could potentially lead to a loss of contraceptive effect.
- Concomitant use with the medicinal products containing ombitasvir/ paritaprevir/ ritonavir and dasabuvir, with or without ribavirin may increase the risk of ALT elevations (See Section Contraindications and Section Warnings and Precautions). Therefore, users must switch to an alternative method of contraception (e.g. progestogen-only contraception or non-hormonal methods) prior to starting therapy with this combination drug regimen. Oralcon can be restarted 2 weeks following completion of treatment with this combination drug regimen.

Dosage and Administration

First treatment cycle: 1 tablet daily for 21 days, starting on the first day of the menstrual cycle. Contraceptive protection begins immediately.

Subsequent cycles: Tablet-taking from the next pack of Oralcon is continued after a 7-day interval, beginning on the same day of the week as the first pack.

Changing from 21-day combined oral contraceptives: The first tablet of Oralcon should be taken on the first day immediately after the end of the previous oral contraceptive course. Additional contraceptive precautions are not required.

Changing from a combined Every Day pill (28 -day tablets):

Oralcon should be started after taking the last active tablet from the Every Day Pill pack. The first Oralcon tablet is taken the next day. Additional contraceptive precautions are not then required.

Changing from a progestogen-only pill (POP):

The first tablet of Oralcon should be taken on the first day of bleeding, even if a POP has already been taken on that day. Additional contraceptive precautions are not then required. The remaining progestogen-only pills should be discarded.

Missed Tablets

Oralcon contains a very low dose of both hormones, and, as a consequence, the contraceptive efficacy margin is small. If a pill is missed. If the woman is less than 12 hours late in taking any tablet, contraceptive protection is not reduced. The woman should take the tablet as soon as she remembers and take the next tablets at the usual time.

If the woman is more than 12 hours late in taking her tablet, contraceptive effectiveness may have decreased. Act according to the following basic rules:

- Never stop taking tablets for a period longer than 7 consecutive days.
- Tablets have to be taken over a sequence of 7 days in order to sufficiently suppress the hypophysis-ovarian axis.

Therefore, the following may be advised in clinical practice.

First week

The woman should take the last tablet she has forgotten as soon as she remembers, even if this means taking two tablets at the same moment. After that, she continues at the usual time with the other tablets. Furthermore, an additional barrier method will have to be used during the next 7 days. If she has had sexual intercourse in the week before she forgot her tablets, possible pregnancy has to be taken into account. The risk of pregnancy is more elevated when forgetting more than 1 tablet and more so when forgetting to take those tablets shortly after the tablet free period.

Second week

The woman should take the tablet she has forgotten as soon as she remembers, even if this means taking two tablets at the same moment. After that, she continues at the usual time with the other tablets. If she has taken her tablets over the previous 7 days correctly, there is no need for additional contraceptive measures. However if more than 1 tablet has been forgotten, it is advisable to use as additional barrier method during the next 7 days.

Third week

Because of the upcoming 7 day tablet free period there is a risk of decreased reliability. The risk can be avoided by adapting the schedule for taking the tablets. If the woman complies with one of the two following rules, there is no need for additional contraceptive measures provided that she has taken her tablets correctly over the seven days previous to forgetting her tablets. If this is not the case, she will have to act according to the first advice given below and also use an additional barrier method for the next 7 days.

- The woman should take the last missed tablet as soon as she remembers, even if this means taking two tablets at the same moment. After that, she continues at the usual time with the other tablets but after taking the last tablet she should continue immediately with the next strip. That is to say there should be no tablet free period. She may not have a withdrawal bleeding until the end of the second strip. She may lose some blood (breakthrough bleeding or spotting) while using the second strip.
- The woman is advised to stop taking tablets from the current strip. She can have a tablet free period of at the most seven days, including those days when she forgot to take her tablets, and start with a new strip after the tablet free period.

In case the woman has no withdrawal bleeding in the next normal tablet free period, the possibility of pregnancy should be taken into account.

Use in Pregnancy

If pregnancy occurs during medication with oral contraceptives, the preparation should be withdrawn immediately (See section 'Reasons for stopping oral contraception immediately').

Use in Lactation

The use of Oralcon during lactation may lead to a reduction in the volume of milk produced and to a change in its composition. Minute amounts of the active substances are excreted with the milk. Mothers who are breast-feeding may be advised instead to use a progestogen-only pill.

Overdosage

Overdosage may cause nausea, vomiting and, in females, withdrawal bleeding. There are no specific antidotes and treatment should be symptomatic. Consult your doctor for any necessary action.

How supplied

Each blister contains 21 white, sugar coated tablets, each containing Levonorgestrel 150 mcg Ph. Eur. and Ethinylestradiol 30 mcg Ph. Eur., followed by pack insert.

Presentation : 3 x 21's and 100 x 21's carton.

Storage

Store below 30°C.

Keep out of reach of children.

Shelf Life: 3 years

Date of Revision: December 2024.



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